

Convalescent Snakes & Ladders: Situating Statement for Play

You have recently been discharged from hospital following a prolonged admission for a serious illness. At the point of discharge, you feel relieved to be home and broadly optimistic. You believe you are recovering well and assume that, with a little rest, life will largely return to how it was before you became ill.

You have been given information about your diagnosis and treatment while in hospital. You know you have a follow-up clinic appointment, but not how it will proceed, and you have had no guidance about what recovery will feel like day to day, what difficulties may occur, or how to interpret any possible setbacks.

Many of your expectations and beliefs about your recovery – what your starting point is, what is normal, what is concerning, and what will improve with time, and when – have not been explicitly confirmed or corrected by a clinician.

From this point on, you are responsible for navigating recovery using the information you have, interpreting changes as they arise, and deciding when uncertainty warrants re-engagement with care.

You are not being asked to solve a problem. You are being asked to proceed.

Note for Facilitators:

Facilitators may adapt the situating statement to align with a particular teaching context or patient population. However, adaptations should avoid detailed clinical vignettes, diagnostic specificity, emotional backstory, or outcome framing. The statement should preserve the core epistemic conditions of discharge: partial information, unvalidated expectations, and responsibility for sense-making without privileged insight.